BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Safety and response tests have different purposes. Use the individual prescription and appointment dates.

Check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal. An electrocardiogram (ECG) (heart tracing) is also a baseline National Cancer Control Programme (NCCP) test.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments	Agree treatment aim, formulation, duration and medicines list.	Swallow tablets whole, with or without food, at about the same time daily. If no more than 12 hours late, take the missed dose when remembered; if more than 12 hours late, skip it and resume the next day. Never double the dose.	Keep an accurate medicine / injection record.	If treatment is interrupted for more than 7 days, contact the team before restarting: a new loading dose is normally needed. Some interacting medicines require a different dose or separation by at least 6 hours. Do not change the dose yourself; ask pharmacy to check every new medicine.	Continue until the team changes the plan.
Cancer response	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00830 v3: official regimen · Orgovyx: product information

Tests in your plan

Blood count, kidney and liver tests

WHEN

Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.

Checks blood cells and organ health.

Blood glucose and bone blood profile

WHEN

Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.

Checks metabolic effects and blood minerals.

PSA and testosterone

If part of your plan

WHEN

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

Checks cancer activity and hormone suppression.

ECG and bone-density assessment

If part of your plan

WHEN

Baseline ECG is required for relugolix. Other ECGs and bone-density scans are based on risk and clinical need.

Checks heart rhythm or fracture risk when indicated.

THE WHOLE TREATMENT JOURNEY

Relugolix: daily hormone treatment: treatment and checks

Adults with advanced hormone-sensitive prostate cancer, following specialist assessment.

Relugolix 360 mg on the first day, then 120 mg once daily without planned breaks. Your specialist sets the duration and any prescribed changes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety NCCP · SMPC	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal. An ECG (heart tracing) is also a baseline NCCP test.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments NCCP · SMPC	Agree treatment aim, formulation, duration and medicines list.	Swallow tablets whole, with or without food, at about the same time daily. If no more than 12 hours late, take the missed dose when remembered; if more than 12 hours late, skip it and resume the next day. Never double the dose.	Keep an accurate medicine / injection record.	If treatment is interrupted for more than 7 days, contact the team before restarting: a new loading dose is normally needed. Some interacting medicines require a different dose or separation by at least 6 hours. Do not change the dose yourself; ask pharmacy to check every new medicine.	Continue until the team changes the plan.
Cancer response NCCP · SMPC	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

What each check is for

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Treatment and review

Loading dose, then once daily

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Ask / act: Repeat blood count, kidney and liver tests and blood glucose as arranged. Bone blood tests and ECG are repeated when clinically indicated. PSA, testosterone and scans follow the individual cancer plan.

Sources: NCCP · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.52